

****DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 47

Age: 77 years old | Sex: Male

MRN: 000-TEST-47

Admission Date: [DATE] | Discharge Date: [DATE]

Attending Physician: Dr. M. Richardson, MD

****PRIMARY DIAGNOSIS & COMORBIDITIES****

- Acute cholecystitis with cholelithiasis — s/p laparoscopic cholecystectomy
- Chronic microcytic anemia (baseline Hgb 9.2 g/dL)
- Chronic pain syndrome (lumbar region)
- HTN, Type 2 DM (non-insulin dependent)

****HOSPITAL COURSE****

Patient 47 presented via ED with acute RUQ pain, fever 101.8°F, and elevated WBC (14.2K). Imaging (US, HIDA scan) confirmed acute cholecystitis with gallstones. Patient taken to OR on hospital day 1. Procedure performed laparoscopically without conversion to open technique. Four trocars placed: umbilical (camera port), epigastric, midclavicular, and anterior axillary (all 5mm). Gallbladder successfully removed; no choledochal stones noted. EBL minimal (<50 mL). Pt tolerated procedure well, extubated in PACU without complications.

Post-op course uncomplicated. Patient advanced diet from NPO→clear liquids (post-op day 1)→full liquids (post-op day 2)→regular diet (post-op day 3) without nausea/vomiting. Ambulating independently. Trocar sites intact, minimal erythema. Foley DC'd post-op day 2. Pain controlled with multimodal regimen.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Acetaminophen	650 mg	TID PRN	2 weeks
Ibuprofen	400 mg	BID with food	1 week
Metformin	1000 mg	BID	Continue indefinitely
Lisinopril	10 mg	q.d.	Continue
Ferrous sulfate	325 mg	1 tab	q.d. Ongoing (anemia)
Omeprazole	20 mg	q.d.	4 weeks

****ACTIVITY & RESTRICTIONS****

- NO lifting >10 lbs × 4 weeks post-op
- NO driving while on narcotic analgesics (none prescribed at discharge)
- Avoid strenuous activity, heavy gardening, or gym-type exercise × 6 weeks
- Light walking encouraged
- May return to sedentary work in 2 weeks; may resume normal duties in 6 weeks pending tolerance

****DIET****

- Regular diet tolerated; advance as tolerated
- Low-fat preferred to minimize GI symptoms initially
- Adequate hydration (8+ glasses water daily)
- No dietary restrictions related to cholecystectomy long-term

****WOUND CARE****

- Keep trocar sites clean & dry; remove bandages in 48 hours
- Shower OK after 48 hours; pat dry gently
- Call if signs of infection: increasing erythema, purulence, warmth, fever >100.4°F
- No tub baths × 2 weeks

****FOLLOW-UP APPOINTMENTS****

1. Surgery Clinic (Dr. Richardson) — in 2 weeks for post-op wound check
2. Primary Care/Internal Medicine — in 3 weeks for anemia reassessment, CBC recheck
3. GI Consultation (optional) — in 6 weeks if persistent abdominal discomfort
